

Constipation in Children

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Constipation is delay or difficulty in passing stool. Stools are harder and sometimes larger than usual and may be painful to pass. Constipation is very common among children. It accounts for up to 5% of children's visits to the doctor.

The frequency and consistency of bowel movements (BMs) vary throughout childhood, and there is no single definition of what is normal. Newborns typically have 4 or more stools per day. During the first year, infants have 2 to 4 BMs a day. Breastfed infants typically have more BMs than formula-fed infants and may have one after each breastfeeding. The stools of breastfed infants are loose, yellow, and seedy. After a month or two, some breastfed infants have BMs less frequently, but the stools remain mushy or loose. After 1 year of age, most children have one or sometimes two soft but formed stools a day. However, some infants and young children typically have BMs only once every 3 to 4 days.

Children may have constipation if they have any of the following:

- No BMs for 2 or 3 more days than usual
- Hard or painful BMs
- Large stools that may clog the toilet
- Drops of blood on the outside of the stool

In infants, signs of effort such as straining and crying before successfully passing a soft stool usually do not indicate constipation. These signs usually occur because infants gradually develop the muscles that are needed to assist a BM.

Parents often worry about their child's BMs, but constipation usually has no serious consequences. Some children with constipation regularly have symptoms of abdominal pain, particularly after meals. Occasionally, passing large, hard stools may cause a small tear in the [anus \(anal fissure\)](#). Anal fissures are painful and may result in streaks of bright red blood on the outside of the stool or on toilet paper. Rarely,

chronic constipation can contribute to urinary problems such as [urinary tract infections](#) and [bed-wetting](#).

(See also [Constipation](#) in adults.)

Causes of Constipation in Children

Common causes

Infants and children are particularly prone to developing constipation at 3 periods of time:

- When [cereals and solid food are introduced](#) into the infant's diet
- During [toilet training](#)
- Around the start of school

In 95% of children, constipation results from

- Dietary issues
- Behavioral issues

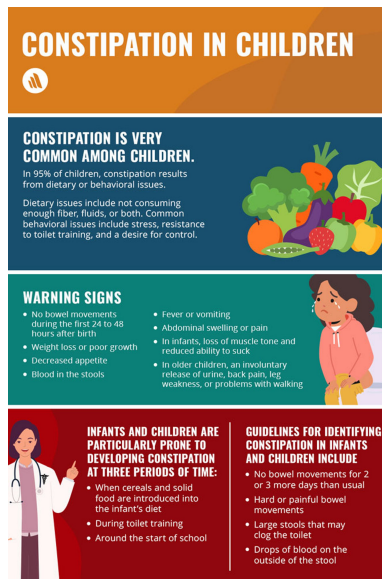
Constipation that results from dietary or behavioral issues is called functional constipation.

Dietary issues that cause constipation include a diet that is low in fluids and/or fiber (fiber is present in fruits, vegetables, and whole grains).

Behavioral issues that may be associated with constipation include stress (as may be felt when a sibling is born), resistance to toilet training, and a desire for control. Also, children may intentionally put off having BMs (called stool withholding) because they have a painful anal fissure or because they do not want to stop playing. [Sexual abuse](#) may result in stress or injury that causes children to withhold stool.

If children do not move their bowels when the natural urge comes, the rectum eventually stretches to accommodate the stool. When the rectum has stretched, the urge to have a BM lessens, and more and more stool accumulates and hardens. A vicious circle of worsening constipation may result. If the accumulated stool hardens, it sometimes blocks the passage of other stool—a condition called fecal impaction. Looser stool from above the hardened stool may leak around the impaction into the child's underwear and lead to [stool incontinence](#) (encopresis). Parents may then think that the child has diarrhea when the actual problem is constipation.

At a Glance: Constipation in Children



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Less common causes

In approximately 5% of children, constipation results from a physical disorder, medication, or toxin. Disorders may be apparent at birth or develop later. Constipation that results from a disorder, medication, or toxin is called organic constipation and needs to be evaluated by a doctor.

In newborns and infants, the most common disorder that causes organic constipation is

- [Hirschsprung disease](#) (an inadequate nerve supply to the large intestine)

Other causes of organic constipation include

- [Birth defects of the anus](#)
- [Cystic fibrosis](#)
- Metabolic and electrolyte disorders, such as an abnormally high level of calcium ([hypercalcemia](#)) or low level of potassium ([hypokalemia](#)) in the blood
- Spinal cord problems (such as [spina bifida](#))
- Hormonal disorders, such as an underactive thyroid gland ([hypothyroidism](#))

- Intestinal disorders, such as a [cow's milk protein allergy](#) or [celiac disease](#)
- Medications, such as powerful pain relievers called [opioids](#) (for example, codeine and morphine)
- Toxins, such as [lead](#) or those that cause [infant botulism](#)

Children with serious abdominal disorders (such as [appendicitis](#) or a [blockage in the intestine](#)) often do not have BMs. However, these children typically have other, more prominent symptoms, such as abdominal pain, abdominal swelling, vomiting, or a combination. These symptoms typically lead parents to seek medical care before the number of BMs decreases.

Evaluation of Constipation in Children

Doctors first try to determine whether constipation results from dietary or behavioral issues (functional) or from a disorder, toxin, or medication (organic).

Warning signs

Certain symptoms are cause for concern and should raise suspicion for an organic cause of constipation:

- No bowel movements (BMs) during the first 24 to 48 hours after birth
- Weight loss or poor growth
- Decreased appetite
- Blood in the stools
- Fever
- Vomiting
- Abdominal swelling
- Abdominal pain (in children old enough to communicate this)
- In infants, loss of muscle tone (the infant appears floppy or weak) and reduced ability to suck
- In older children, an involuntary release of urine ([urinary incontinence](#)), back pain, leg weakness, or problems with walking

When to see a doctor

Children should be evaluated by a doctor right away if they have any warning signs. If no warning signs are present but the child is passing infrequent, hard, or painful BMs, a doctor should be called. Depending on the child's other symptoms (if any), the

doctor may advise trying simple home [treatments](#) or ask the parents to bring in the child for an examination.

What the doctor does

Doctors first ask questions about the child's symptoms and medical history. Doctors then do a physical examination. What they find during the history and physical examination often suggests a cause of the constipation and the tests that may need to be done (see table [Some Physical Causes and Features of Constipation in Infants and Children](#)).

Doctors determine whether newborns have ever had a BM (the first BM is called meconium). Newborns who have not had a BM within 24 to 48 hours after birth should have a thorough examination to rule out the possibility of Hirschsprung disease, anorectal malformations, or other serious disorder.

For infants and older children, doctors ask whether constipation began after a specific event, such as introducing cereal or other solid foods, eating honey (in children under 12 months of age), beginning toilet training, or starting school. For all age groups, doctors ask about diet and about disorders, toxins, and medications that can cause constipation.

For the physical examination, doctors first look at the child overall for signs of illness and measure height and weight to check for signs of delayed growth. Doctors then focus on the abdomen, the anus (including examination of the rectum using a gloved finger), and nerve function (which can affect how the digestive tract functions).

Testing

If the cause of constipation appears to be functional, no tests are needed unless children are not helped by treatment. If treatment does not help or if doctors suspect the cause is another disorder, an [x-ray](#) of the abdomen may be taken, and tests for other disorders are done based on the results of the examination.



Some Physical Causes and Features of Constipation in Infants and Children

Cause	Common Features*	Tests
Birth defects of the anus		
Abnormal position of the anus	Opening of the anus that appears closer than normal to the genitals	Measurements to determine the exact location of the anus's opening
Anal stenosis (a narrowed anus)	Delayed passage of the first BM (called meconium) during the first 24–48 hours of life Explosive and painful BMs A swollen abdomen Abnormal appearance or position of the anus	A doctor's examination
Blockage of the opening of the anus (anorectal malformations)	A swollen abdomen No BMs A blockage of the anus detected during a doctor's examination	A doctor's examination done soon after birth
Spinal cord problems		
Meningomyelocele (the most severe form of spina bifida)	A raw, red area on the back where the spinal cord protrudes, seen at birth A decrease in reflexes of the legs or in muscle tone Absence of the normal reflex of the anus (a	Plain x-rays of the lower spine MRI of the spine



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